



EVALUATION 1 — HIGH ACUITY

Primary Diagnosis: Acute Exacerbation of Congestive Heart Failure (NYHA Class III)

PATIENT DEMOGRAPHICS

Field Detail

Patient Name Margaret Louise Finnegan

Date of Birth 03/14/1941

Age 84

Sex Female

Gender Identity Female (she/her)

Race/Ethnicity White / Non-Hispanic

Primary Language English

Interpreter Needed No

Marital Status Widowed

Social Security # (last 4) XXX-XX-7823

Medical Record Number CI-2025-00417

Address of Service 1847 Oleander Court, Unit B, San Mateo, CA 94401

Home Phone (650) 555-0193

Cell Phone (650) 555-0247

Email None

County San Mateo

EMERGENCY CONTACTS

Priority Name Relationship Phone (Primary) Phone (Alt) Lives with Patient?

1 Robert Finnegan Son (650) 555-0381 (415) 555-0294 No — lives 12 miles away

2 Diana Finnegan-Marsh Daughter (510) 555-0762 (510) 555-0148 No — lives 38 miles away

3 Patricia Yuen Neighbor / Friend (650) 555-0455 N/A No — lives next door, Unit A

LIVING SITUATION

Field Detail

Lives With Alone

Residence Type Single-story duplex, rented

Stairs to Enter 2 steps to front door (no handrail)

Interior Stairs None

Elevator N/A

Pets 1 indoor cat ("Biscuit") — elderly, not aggressive

Home Access Instructions Lockbox on front door handle; code to be provided to care team by son Robert

DME in Home Hospital bed (rented), bedside commode, rolling walker, portable oxygen concentrator (5 LPM)

PAYER INFORMATION

Priority Payer ID / Policy Number Group Number Effective Date
Primary Medicare Part A (Traditional) 1EG4-TE5-MK72 N/A 07/01/2014
Secondary Medi-Cal (Fee-for-Service) 94038712A N/A 01/01/2019
Tertiary N/A
Private Pay: No Workers' Comp: No VA/TRICARE: No

PHYSICIAN INFORMATION

Role Name Specialty Phone Fax NPI Address
Attending / Certifying Physician Dr. Alan K. Chowdhury Internal Medicine (650) 555-0612 (650) 555-0613 1234567890 2100 Bay Medical Plaza, Ste 310, San Mateo, CA 94401
Cardiologist Dr. Renata Silva-Gutierrez Cardiology (650) 555-0820 (650) 555-0821 2345678901 2100 Bay Medical Plaza, Ste 445, San Mateo, CA 94401
PCP (if different) Same as attending
Face-to-Face Encounter Date: 06/18/2025 F2F Performed By: Dr. Alan K. Chowdhury F2F Encounter Findings: Patient seen in office; significant dyspnea on minimal exertion (walking >15 ft), bilateral lower extremity edema (+2 pitting), weight gain 8 lbs in 10 days, oxygen saturation 89% on room air at rest. Requires skilled nursing for CHF management, medication teaching, daily weight monitoring, and oxygen management. Homebound due to severe dyspnea and functional limitation.

REFERRAL / HOSPITALIZATION HISTORY

Event Facility Dates Reason
Hospital Discharge San Mateo Medical Center 06/12/2025 – 06/17/2025 Acute CHF exacerbation, fluid overload, IV diuresis
ED Visit San Mateo Medical Center 05/01/2025 Syncope — evaluated and released
Hospital Admission San Mateo Medical Center 02/08/2025 – 02/12/2025 CHF exacerbation, pneumonia
Hospital Admission Sequoia Hospital 11/15/2024 – 11/19/2024 CHF exacerbation with atrial fibrillation RVR
Start of Care Date: 06/19/2025 Referral Source: San Mateo Medical Center Discharge Planning — Case Manager: Linda Torres, RN, (650) 555-0399

DIAGNOSES

Priority ICD-10 Code Diagnosis Onset
Primary I50.33 Acute on chronic diastolic (congestive) heart failure, NYHA Class III 06/12/2025
Secondary I48.91 Unspecified atrial fibrillation (chronic, rate-controlled) 2018
Secondary I10 Essential (primary) hypertension 2005
Secondary E11.65 Type 2 diabetes mellitus with hyperglycemia 2010
Secondary N18.3 Chronic kidney disease, stage 3 (GFR 38 mL/min, 06/2025) 2020
Secondary E78.5 Dyslipidemia, unspecified 2008
Secondary J44.1 Chronic obstructive pulmonary disease with acute exacerbation (resolved, history of) 2022
Secondary M81.0 Age-related osteoporosis without current pathological fracture 2019
Secondary G47.33 Obstructive sleep apnea 2017
Secondary F32.1 Major depressive disorder, single episode, moderate 2023
Secondary H25.13 Age-related nuclear cataract, bilateral 2021

Surgical History

Right total knee arthroplasty (2016)
Cholecystectomy (2003)
Hysterectomy, total abdominal (1998)
Cataract extraction, left eye (2022)

ALLERGIES

Allergen Type Reaction Severity

Penicillin Drug Anaphylaxis (throat swelling, hypotension) — documented 1987 SEVERE

Sulfa drugs Drug Diffuse maculopapular rash, pruritus Moderate

Iodine contrast dye Contrast Urticaria, facial swelling Moderate

Latex Environmental Contact dermatitis Mild

Shellfish Food Nausea, abdominal cramping Mild

NKDA: ☐ No known drug allergies — NOT applicable; see above

CURRENT MEDICATIONS (as of discharge 06/17/2025)

Medication Dose Route Frequency Prescriber Purpose High-Risk?

1 Furosemide (Lasix) 80 mg PO BID (8 AM, 2 PM) Chowdhury Diuresis / CHF Yes

2 Potassium Chloride (K-Dur) 20 mEq PO BID Chowdhury K+ replacement (with diuretic) Yes

3 Carvedilol (Coreg) 12.5 mg PO BID Silva-Gutierrez Heart failure / rate control Yes

4 Lisinopril 10 mg PO Daily (AM) Chowdhury HTN / CHF / renal protection Yes

5 Warfarin (Coumadin) 4 mg PO Daily (6 PM) Silva-Gutierrez Anticoagulation (A-fib) YES — HIGH ALERT

6 Digoxin (Lanoxin) 0.125 mg PO Daily (AM) Silva-Gutierrez A-fib rate control / CHF YES — HIGH ALERT

7 Metformin 500 mg PO BID (with meals) Chowdhury Type 2 DM Yes

8 Atorvastatin (Lipitor) 40 mg PO Daily (HS) Chowdhury Dyslipidemia No

9 Sertraline (Zoloft) 100 mg PO Daily (AM) Chowdhury Depression No

10 Albuterol (ProAir HFA) 2 puffs INH Q4-6H PRN Chowdhury Bronchospasm / COPD No

11 Alendronate (Fosamax) 70 mg PO Weekly (Sunday AM, 30 min before food/meds) Chowdhury Osteoporosis No

12 Calcium + Vitamin D 600 mg / 400 IU PO BID Chowdhury Bone health No

13 Docusate Sodium (Colace) 100 mg PO BID Chowdhury Constipation prophylaxis No

14 Acetaminophen (Tylenol) 650 mg PO Q6H PRN Chowdhury Pain (knee, general) No

15 Oxygen 2-3 LPM Nasal cannula Continuous at rest; 3 LPM with activity Chowdhury Hypoxemia / CHF / COPD Yes

Medication Reconciliation Completed: ☐ Yes — by admitting RN on _____ Discrepancies Identified: ☐ Yes ☐ No (if yes, document and notify physician) Controlled Substances in Home: None Insulin in Home: No Sharps in Home: No Medication Organizer Used: Yes — 7-day AM/PM pill organizer (filled by son Robert weekly)

CLINICAL ASSESSMENT — VITAL SIGNS AT ADMISSION

Parameter Value Date/Time Normal Range Abnormal?

Blood Pressure 158/92 mmHg 06/19/2025 10:30 AM <130/80 Yes — elevated

Heart Rate 88 bpm, irregularly irregular 06/19/2025 10:30 AM 60-100 WNL (irregular rhythm — known A-fib)

Respiratory Rate 22 breaths/min 06/19/2025 10:30 AM 12-20 Yes — mildly elevated

Temperature 98.2°F (oral) 06/19/2025 10:30 AM 97.0-99.0 WNL

Oxygen Saturation (SpO2) 91% on 2 LPM O2 06/19/2025 10:30 AM ≥95% Yes — low

SpO2 on Room Air 87% (desaturated within 45 seconds of O2 removal) 06/19/2025 10:35 AM ≥95% Yes — significantly low

Weight 178 lbs (80.7 kg) 06/19/2025 10:25 AM Baseline ~165 lbs pre-exacerbation Yes — 13 lbs above baseline

Height 5'4" (162.5 cm) Per history

BMI 30.5 Calculated 18.5-24.9 Yes — obese class I

Pain 4/10 (bilateral knee pain, worse with standing) 06/19/2025 0 = no pain Yes

Blood Glucose 188 mg/dL (fasting) 06/19/2025 08:15 AM (patient report) 70-130 fasting Yes — elevated

CLINICAL ASSESSMENT — SYSTEMS REVIEW

Cardiovascular

History of CHF (diastolic, NYHA Class III at present), chronic atrial fibrillation (rate-controlled), hypertension.
Currently on anticoagulation with warfarin; INR due for check (last INR 2.3 on 06/16/2025 — inpatient).
Bilateral lower extremity edema: +2 pitting, bilateral, to mid-shin. Skin intact, no weeping.
JVD noted at 45 degrees.
S3 gallop appreciated on auscultation.
Peripheral pulses: dorsalis pedis 1+ bilaterally, posterior tibial 1+ bilaterally.
Capillary refill: 4 seconds bilaterally (delayed).
No calf tenderness, no Homans' sign.
Daily weight monitoring ordered — patient has digital scale in bathroom.

Respiratory

History of COPD (mild-moderate, FEV1 62% predicted on last PFT 2023).
Oxygen-dependent at rest (2 LPM) and with activity (3 LPM).
Bilateral basilar crackles (fine), more prominent on right than left — consistent with residual fluid.
Dyspnea on exertion: onset with walking >10-15 feet; relieved with rest and supplemental O2 increase.
Dyspnea at rest: intermittent, particularly when supine (orthopnea — sleeps with 3 pillows elevated).
Productive cough: small amounts of white/frothy sputum, non-purulent.
No hemoptysis.
CPAP for OSA — patient reports non-compliance x 6 months ("the mask bothers me").

Neurological / Cognitive

Alert and oriented x4 (person, place, time, situation).
MMSE-equivalent screening: 26/30 (missed serial 7s, missed date by 1 day, difficulty with 3-item recall at 5 minutes — 1/3 recalled).
Speech clear, appropriate, coherent. No aphasia or dysarthria.
Cranial nerves grossly intact.
Bilateral upper extremity strength 4/5 (mild generalized weakness post-hospitalization).
Bilateral lower extremity strength 3+/5 (weakness, deconditioning, knee pain).
Sensation intact to light touch bilaterally — diminished at toes bilaterally (diabetic neuropathy suspected).
No tremor, no focal deficits.
Visual acuity: diminished (cataracts bilateral, L > R; uses reading glasses). Difficulty reading medication bottles.
Hearing: mild bilateral sensorineural hearing loss — no hearing aids, denies need but frequently asks for repetition.

Gastrointestinal

Abdomen soft, non-tender, non-distended. Bowel sounds active x4 quadrants.
Last BM: 06/18/2025 — soft, brown, no blood.
Dentures: upper plate (fits well); lower natural teeth (several missing).
Appetite: poor since hospitalization; estimates eating 40-50% of meals.
Diet order: low-sodium (2g Na+/day), diabetic-consistent, fluid restriction 1.5L/day.
Reports difficulty preparing meals; son Robert brings meals 3x/week, otherwise relies on frozen meals (high sodium concern).

Genitourinary

Voiding independently; nocturia x3-4 (likely related to diuretic timing and CHF).
No incontinence. No dysuria, no hematuria.
No catheter. No history of UTI in past 12 months.
CKD Stage 3 — last creatinine 1.8, BUN 34, GFR 38 (06/15/2025 inpatient labs).
Nephrology referral pending.

Integumentary / Skin

Skin generally intact.
Bilateral lower extremities: +2 pitting edema, skin taut and shiny, no breakdown, no cellulitis. Hemosiderin staining bilateral medial ankles (chronic venous changes).
Sacrum/coccyx: intact, no redness, no pressure injury. Braden Scale: 16 (mild risk).
Bilateral knees: well-healed surgical scar right knee (TKA 2016); no erythema, warmth, or drainage.
No wounds, no ulcers, no skin tears.
Skin turgor: mildly decreased (upper extremities) — may indicate suboptimal hydration despite fluid overload.

Musculoskeletal / Functional

Transfers: bed to chair with moderate assist of 1 and use of bedrail. Stands with rolling walker and moderate assist of 1.
Ambulation: rolling walker, 10-15 feet maximum before significant dyspnea and fatigue. Requires contact guard to standby assist.
Stairs: unable at this time (2 steps to front door — uses single handrail + assist of 1).

ADLs

Bathing: requires moderate assist (seated shower, uses shower chair)
Dressing upper body: minimal assist (difficulty with overhead motion due to fatigue)
Dressing lower body: moderate assist (cannot bend to put on shoes/socks due to edema, dyspnea)
Toileting: uses bedside commode at night; ambulates to bathroom with walker during day (standby assist)
Eating: independent (when food is prepared and placed within reach)
Grooming: independent (seated)

IADLs

Meal preparation: unable (fatigue, dyspnea, standing tolerance <3 minutes)
Medication management: requires supervision/assist (poor understanding of regimen changes post-discharge; visual impairment limits label reading)
Telephone use: independent
Laundry: unable
Light housekeeping: unable
Shopping: unable
Transportation: unable to drive; dependent on son or paratransit

Psychosocial / Emotional

PHQ-9 Score: 14 (moderate depression)

Reports feeling "useless" and "like a burden to my kids."

Widowed x 7 years; reports ongoing grief, worsened with repeated hospitalizations.

Social isolation: lives alone, limited visitors (son 2-3x/week; daughter monthly). Neighbor Patricia checks in daily by phone.

Previous psychotherapy: none. Open to MSW support "if it would help."

Denies suicidal ideation, homicidal ideation, or self-harm.

Denies alcohol, tobacco (quit 1995), or illicit drug use.

Spiritual: Catholic; parish priest visits monthly. Faith is important to her.

Reports anxiety about falling when alone, especially at night.

HOMEBOUND STATUS ASSESSMENT

Criterion Finding

Criterion 1 — Considerable and taxing effort to leave home MET. Patient develops significant dyspnea (SpO2 drops to 87% on RA) with ambulation >10-15 feet. Requires rolling walker + assist of 1 to ambulate. Unable to navigate 2 steps to front door without assist. Severe fatigue with minimal activity. Bilateral LE edema limits mobility.

Criterion 2 — Absences from home are infrequent/short/for medical care MET. Patient has not left home since hospital discharge 06/17/2025. Son drives her to medical appointments only. No other outings planned or feasible.

Homebound Status CONFIRMED — homebound

ADVANCE DIRECTIVES

Document Status On File? Date Executed

Advance Health Care Directive (AHCD) Yes — executed ☐ Copy provided at admission 03/2021

Power of Attorney for Health Care Robert Finnegan (son) designated as agent Included in AHCD 03/2021

POLST Yes — completed ☐ Copy provided at admission 06/2025 (updated during hospitalization)

POLST Orders Full treatment (Section A); selective treatment including hospitalization but no intubation (Section B); trial period of artificial nutrition (Section D)

DNR No — patient is FULL CODE per POLST Section A

Mental Health Directive None

HOME SAFETY ASSESSMENT

Area Finding Recommendation Patient Response

Entry 2 steps, no handrail on left side Install grab rail left side of steps Son Robert will install this weekend

Lighting — Entry Dim; single 40W bulb Replace with higher-wattage LED bulb Agreed

Lighting — Hallway to Bathroom Adequate None N/A

Lighting — Bedroom Nightlight present but burned out Replace nightlight Agreed

Flooring 2 loose area rugs in living room; 1 bath mat (non-slip backing worn) Remove area rugs; replace bath mat Patient reluctant about rugs ("they make it feel like home") — educated on fall risk, will discuss with son

Bathroom Shower chair present (stable); no grab bars near toilet or shower; bath mat worn Install grab bars at toilet and inside shower; replace bath mat Son will arrange installation

Bedroom Hospital bed present (functional); bedside commode in place; path to bathroom clear; 3 pillows for orthopnea positioning None — adequate N/A

Kitchen Moderate clutter on counters; no fire extinguisher; smoke detector present (functional) Reduce clutter; obtain fire extinguisher; educate on stove safety Agreed to clutter reduction; son will bring extinguisher

Smoke Detectors 1 in hallway (functional; battery tested); none in bedroom or kitchen Install smoke detectors in bedroom and kitchen Son will install

Carbon Monoxide Detector None Install CO detector near bedroom Son will purchase and install

Medications Stored in kitchen cabinet; not organized; multiple expired medications from prior regimens mixed in; Coumadin not separated from other meds Organize medications; discard expired meds; separate high-alert medications; place medication list on refrigerator; consider locking med box for Coumadin RN will organize at next visit with patient/son

Oxygen Portable concentrator present; tubing adequate length (25 ft); no open flames observed; patient aware of O2 safety (verbal confirmation); no "No Smoking" sign posted Post "Oxygen in Use — No Smoking" signs at front door and in bedroom RN will provide signs at next visit

Sharps None in home at this time N/A N/A

Emergency Numbers Not posted Post emergency numbers (911, CI after-hours, son Robert, Dr. Chowdhury) on refrigerator RN will provide card at next visit

Phone Access Cordless landline in living room; cell phone usually on nightstand Ensure phone within reach at all times, including bathroom trips Agreed — will keep cell phone in robe pocket

Weapons Patient denies firearms or weapons in home None N/A

Pets 1 indoor cat — elderly, calm, not a tripping hazard; litter box in laundry room (son cleans weekly) Monitor cat litter area for fall risk N/A

General Cleanliness Adequate; mild clutter in living room (magazines, mail) Reduce clutter in walkways Agreed

Temperature Central heating/cooling present; thermostat set at 72°F; adequate None N/A

Water Temperature Tested at bathroom sink: 118°F Within safe range (<120°F) N/A

Overall Home Safety Risk Level: Moderate Immediate Hazards Identified: Loose area rugs (fall risk); absent grab bars at toilet/shower; expired medications mixed with current; no CO detector; no bedroom smoke detector.

SERVICES ORDERED

Service Discipline Frequency Duration Start Date

Skilled Nursing RN 3x/week x 3 weeks, then 2x/week x 3 weeks, then 1x/week x 2 weeks 8 weeks (anticipated) 06/19/2025

Physical Therapy PT 3x/week x 4 weeks, then 2x/week x 4 weeks 8 weeks (anticipated) 06/20/2025

Occupational Therapy OT 2x/week x 3 weeks, then 1x/week x 3 weeks 6 weeks (anticipated) 06/20/2025

Medical Social Work MSW 1x/week x 4 weeks, then PRN 4 weeks + PRN 06/23/2025

Home Health Aide HHA 3x/week (bathing assist, ADL support) Duration of need 06/23/2025

SKILLED NURSING CARE PLAN FOCUS AREAS

CHF Management & Monitoring

Daily weight monitoring — patient/caregiver education on technique, timing (AM, after void, before breakfast, same clothing), and documentation
Teach CHF "zone" system: Green zone (stable), Yellow zone (call MD — weight gain >2 lbs/day or >5 lbs/week, increased swelling, increased dyspnea), Red zone (call 911 — chest pain, severe SOB, fainting)
Assess bilateral lower extremity edema each visit — measure circumference at ankle and mid-calf
Monitor JVD, lung sounds (crackles), and SpO2 each visit
Reinforce fluid restriction (1.5L/day) — teach fluid measurement and tracking
Reinforce low-sodium diet (2g/day) — provide written materials, review food labels, evaluate frozen meal sodium content
Educate on sodium hidden sources (canned soups, frozen meals, condiments, bread)
Medication Management

Complete medication reconciliation — compare discharge summary, pharmacy bottles, and pill organizer
Dispose of expired/discontinued medications safely
Organize current medications with son/patient
Teach purpose, dose, frequency, timing, side effects, and interactions for all medications, with emphasis on:
Warfarin: INR monitoring schedule, signs of bleeding (bruising, gum bleeding, blood in stool/urine, prolonged wound bleeding, headache), dietary consistency (Vitamin K foods), drug interactions (OTC meds, herbals), lab schedule
Digoxin: teach to check pulse before taking (hold and call MD if <60 or >110 or irregular change), signs of toxicity (nausea, vomiting, visual disturbances, confusion, bradycardia)
Furosemide: timing (not after 2 PM to reduce nocturia), electrolyte monitoring, signs of dehydration vs. fluid overload, importance of potassium supplementation
Carvedilol: orthostatic hypotension precautions, do not abruptly discontinue
Lisinopril: cough as common side effect, report angioedema immediately
Metformin: take with meals, hold if NPO or having contrast procedures, signs of lactic acidosis (rare — muscle pain, weakness, difficulty breathing)
Coordinate with son Robert re: weekly pill organizer filling
Assess need for pharmacy medication synchronization (med sync) program
INR monitoring: draw or coordinate INR draw per physician-specified schedule (goal INR 2.0-3.0 for A-fib)
Oxygen Management

Assess O2 compliance — patient reports removing O2 at night ("it dries out my nose")
Educate on importance of continuous O2 use
Recommend humidifier bottle for concentrator
Verify O2 flow rate matches orders (2 LPM rest, 3 LPM activity)
Assess equipment function at each visit
Monitor SpO2 at each visit (on and off O2 if tolerated)
Reinforce O2 safety (no open flame, no smoking, post signs)
Diabetes Monitoring

FSBS monitoring: patient currently not testing at home (glucometer present but "I forget")
Reinforce daily AM fasting FSBS and PRN if symptomatic
Review FSBS targets per physician (goal 100-180 for this patient given age and comorbidities)
Teach hypo/hyperglycemia signs and management
A1c monitoring: last A1c 7.8% (05/2025) — coordinate with MD for follow-up
Renal Function Monitoring

Monitor for signs of CKD progression: decreased urine output, increased edema, new-onset confusion, nausea
Coordinate lab draws per MD order (BMP/CMP frequency)
Educate patient on CKD and relationship to CHF and DM
Support nephrology referral follow-up
Fall Prevention

Assess fall risk at each visit (current risk: HIGH — history of syncope 05/2025, LE edema, orthostatic BP changes on carvedilol, visual impairment, nocturia, deconditioning)
Reinforce safe transfer techniques
Reinforce call light/phone within reach at all times
Coordinate with PT/OT on fall prevention program
Educate on orthostatic hypotension precautions (rise slowly, sit on edge of bed before standing)
Wound / Skin Surveillance

No wounds currently — monitor for breakdown given edema, CKD, DM, and reduced mobility
Braden Scale reassessment each visit
Educate on skin inspection, moisturizing, and pressure relief
Educate on diabetic foot care (daily inspection, proper footwear, no walking barefoot, no heating pads on feet)
Depression Monitoring

PHQ-9 reassessment every 2 weeks
Assess response to sertraline (started 2023)
Monitor for suicidal ideation
Coordinate with MSW for psychosocial support
Encourage social engagement, activity tolerance progression
Patient/Caregiver Education

All of the above
When to call the MD vs. when to call 911 — provide written wallet card
Teach-back for all education points (CHF zones, weight monitoring, fluid restriction, medications, O2, diet)
Assess and address health literacy limitations (visual impairment, hearing impairment)

PHYSICAL THERAPY EVALUATION FOCUS AREAS

Functional Mobility Assessment

Timed Up and Go (TUG): ____ seconds (anticipated >20 seconds = high fall risk)
30-Second Chair Stand Test: ____ (anticipated <8 = below norm for age)
6-Minute Walk Test: defer until functional status allows; use timed distance test (e.g., 2-minute walk)
Berg Balance Scale: anticipated score <40 (fall risk)
Strengthening Program

Progressive lower extremity strengthening (seated → standing → ambulatory exercises)
Core stabilization
Emphasis on functional task training (sit-to-stand, bed mobility, toilet transfers)
Endurance / Activity Tolerance

Graded activity progression with SpO2 and HR monitoring
Teach Borg Dyspnea Scale for self-monitoring
Collaborate with RN on O2 titration during activity
Gait Training

Rolling walker assessment and training (proper height, technique, turning)
Stair training (2 steps with rail) when safe and appropriate
Obstacle navigation in home
Fall Prevention Program

Balance exercises (static → dynamic)
Environmental modifications (coordinate with OT)
Education on fall recovery techniques

OCCUPATIONAL THERAPY EVALUATION FOCUS AREAS

ADL Training

Energy conservation techniques for bathing, dressing, meal preparation
Adaptive equipment assessment (long-handled shoehorn, sock aid, reacher, tub bench)
Seated task training for kitchen activities
Home Modification Recommendations

Grab bar placement (toilet, shower)
Raised toilet seat assessment
Kitchen organization for accessibility
Medication Management Support

Large-print medication list
Medication organizer setup with visual aids
Adaptive strategies for visual impairment (magnifying glass, contrasting labels)
Energy Conservation / Work Simplification

Activity pacing for CHF management
Teach diaphragmatic breathing during ADLs
Reorganize home to minimize unnecessary ambulation

MEDICAL SOCIAL WORK FOCUS AREAS

Psychosocial Assessment & Support

Depression management — coping strategies, grief counseling referral
Social isolation intervention — community resources, senior center (when mobility improves), phone-based support groups, parish visitor program
Caregiver burden assessment (son Robert)
Community Resources

Meals on Wheels referral (address nutritional needs + sodium-controlled options)
Paratransit enrollment for medical appointments
PG&E Medical Baseline Program enrollment (O2 concentrator)
County emergency services registration
Catholic Charities or parish-based support services
Financial / Insurance Counseling

Verify Medicare/Medi-Cal dual-eligible benefits
Assist with prescription assistance programs if needed
Health Insurance Counseling & Advocacy Program (HICAP) referral for benefits review
Advance Care Planning Support

Review existing AHCD and POLST — confirm patient understands and wishes are current
Facilitate family discussion about goals of care if patient desires

HOME HEALTH AIDE CARE PLAN

Task Frequency Specific Instructions

Bathing Assist 3x/week Seated shower using shower chair; patient washes upper body, aide assists with back, lower body, feet, hair. Water temperature check before bathing. Observe skin for breakdown, redness, new bruising.

Dressing Assist 3x/week (post-bath) Assist with lower body dressing (socks, shoes — use adaptive equipment as trained by OT). Patient independent with upper body dressing (minimal assist as needed).

Personal Care 3x/week Nail care (file only — no cutting due to DM and Coumadin); skin moisturizing (lower legs, feet — avoid between toes); oral care assist if needed.

Vital Signs Each visit Weight, BP, HR, SpO2. Report to RN: weight gain >2 lbs since last visit, SBP >160 or <90, HR <60 or >110, SpO2 <90%.

Ambulation Assist Each visit Assist with ambulation using rolling walker as tolerated. Monitor for dyspnea, fatigue, dizziness.

Light Meal Prep Each visit (as time allows) Prepare simple meals per dietary restrictions (low sodium, diabetic). Assist with fluid measurement/tracking.

Environmental Observations Each visit Report any safety hazards, changes in home environment, or changes in patient behavior/cognition to RN supervisor.

HHA Supervision: RN supervisory visit every 14 days per 42 CFR §484.80(h). RN must make an on-site visit to patient's home during an HHA visit at least once every 14 days.

PLAN OF CARE GOALS (60-DAY CERTIFICATION PERIOD)

#	Goal Area	Measurable Goal	Target Date
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1	CHF Management	Patient/caregiver will verbalize understanding of CHF zones and demonstrate daily weight monitoring with 90% accuracy over 2 consecutive weeks	08/01/2025
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2	CHF — Fluid Status	Patient will achieve dry weight of ≤168 lbs (return toward baseline 165 lbs) with no rehospitalization	08/17/2025
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3	Medication Management	Patient/caregiver will correctly identify all medications by name, purpose, dose, and key side effects using teach-back with ≥80% accuracy	07/31/2025
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4	INR Stability	INR will remain within therapeutic range (2.0-3.0) for ≥80% of draws during episode	Ongoing
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5	Oxygen Compliance	Patient will demonstrate continuous O2 use as ordered, including during sleep, with ≥90% compliance per self-report and equipment use data	07/31/2025
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6	SpO2 Improvement	SpO2 will be ≥92% on prescribed O2 at rest consistently	08/17/2025
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7	Functional Mobility	Patient will ambulate 100 feet with rolling walker and standby assist with SpO2 ≥90%	08/17/2025
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8	Transfer Independence	Patient will transfer bed-to-chair with supervision only (no physical assist)	08/01/2025
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9	Fall Prevention	Zero falls during episode of care	08/17/2025
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10	ADLs	Patient will perform upper body dressing independently and lower body dressing with minimal assist	08/17/2025
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11	Nutrition	Patient will verbalize understanding of low-sodium, diabetic-consistent diet and demonstrate ability to identify high-sodium foods with ≥80% accuracy	07/31/2025
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12	Depression	PHQ-9 score will decrease to ≤10 (from 14)	08/17/2025
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13	Diabetes	Fasting blood glucose will be within target range (100-180) for ≥70% of readings	08/17/2025
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14	Skin Integrity	Skin will remain intact with no new pressure injuries, skin tears, or breakdown	08/17/2025
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15	Home Safety	All identified safety hazards will be remediated (grab bars, rugs, smoke/CO detectors, medication organization)	07/15/2025
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DISCHARGE CRITERIA (ANTICIPATED)

Patient will be considered for discharge when

CHF is compensated and stable (at or near dry weight, no signs/symptoms of decompensation x 2 consecutive weeks).
Medications are reconciled, organized, and patient/caregiver demonstrates competence in medication management.
INR is stable within therapeutic range.
O2 requirements are stable and patient is compliant.
Functional goals are met or plateau is reached.
Patient/caregiver verbalizes understanding of disease management, dietary restrictions, activity guidelines, and when to seek medical attention.
Home environment is safe.
Community resources are in place (Meals on Wheels, paratransit, etc.).
Follow-up appointments are scheduled with cardiologist, nephrologist, and PCP.
Emotional status is improved and support systems are engaged.
Anticipated Discharge Date: 08/17/2025 (end of first 60-day certification period) — subject to reassessment.

PATIENT / REPRESENTATIVE SIGNATURES — EVALUATION 1

I confirm that the information provided above is accurate and complete to the best of my knowledge. I have participated in the development of this plan of care and agree to the services described.

Patient Signature: _____ Date: _____ Time: _____

Print Name: Margaret Louise Finnegan

Authorized Representative Signature (if applicable): _____

Print Name: Robert Finnegan

Relationship: Son / Health Care Agent (POA)

Witness / Admitting Clinician Signature: _____ Date: _____

Print Name / Title / Credentials: _____

Patient / Representative Signature

Date

RN Clinician Signature

Date / Time